A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Sandra Farag
Bluff Road Medical

Dear Dr Farag,

Re: Mr Ryan Djurovich
DOB: 1989-02-02
Address: 1/53A Tibrockney Street, HIGHETT, VIC, 3190

Summary

- **Colonoscopy revealing multiple SSA, including requiring piecemeal resection.**
- **Gastroscopy excluding Barrett's oesophagus and intestinal metaplasia.**
- **For repeat colonoscopy in 12 months to ensure clearance and monitor for polyposis.**
- **Trial of proton pump inhibitor (PPI) cessation with Gaviscon for breakthrough symptoms.**
- **No action required by you.**

I had the pleasure of conducting a review with Ryan Djurovich following his recent gastroscopy and colonoscopy.

The colonoscopy revealed a significant number of precancerous polyps (SSA's). While these were benign, the volume of polyps is atypical for Ryan's age group. One lesion in particular was larger than anticipated and required piecemeal resection to ensure complete removal. Given the family history and the number of polyps identified, I have discussed the possibility of a polyposis syndrome. While we do not yet have sufficient evidence to formally apply this diagnosis, his future surveillance will be critical in determining if there is a genetic predisposition.

Ryan's gastroscopy was reassuring. Biopsies taken from the gastro-oesophageal junction confirmed the absence of Barrett's oesophagus or intestinal metaplasia, as was previously identified. Although there was some evidence of mild inflammation, there was no evidence of malignancy. Ryan is interested in weaning off his PPI medication. We discussed the likelihood of

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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
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St Vincent's Private Hospital
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rebound acid hypersecretion over the first two weeks, and I have advised he use Gaviscon as required for breakthrough symptoms.

Regarding his symptoms, Ryan continues to experience fluctuating bowel habits and occasional loose stools. We explored the multifactorial nature of these symptoms, noting the influence of stress, dietary variables, and his high level of somatic vigilance. I have reassured him that while these symptoms are frustrating, they do not correlate with adverse longevity outcomes. From a metabolic standpoint, we discussed the importance of managing visceral fat and fatty liver through his current exercise and high-fibre diet, which also serves to mitigate his elevated colorectal cancer risk.

Impression and Plan

I have recommended a repeat colonoscopy in 12 months, ideally within this calendar year, to ensure the large polyp site is clear and to monitor the rate of new polyp formation. A surveillance gastroscopy is not required for another two to three years. Ryan will continue with his high-fibre intake and lifestyle modifications. He is cleared to continue using creatine supplements as desired. I have reassured him that his physiological gastrointestinal responses are likely linked to a heightened brain-gut sensitivity rather than underlying pathology. I will review him again following his repeat procedure.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

DJUROVICH, RYAN ASHLEY : SVP 873729

DOB: 02/02/1989 Age: 37 years

Unit 1, 53a Tibrockney St, Highett 3190

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Results from 11/04/2026 ..... Episode# 26P258057 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009388 **CLINICAL NOTES** PHx of IM at GOJ and polyps. Some loose stools. Duo = N. Gas = N. GOJ-on gastric side below z line. Looks like before.? Gastric IM. Lower oesophagus = N. TV polyps x3 = ##Illegible## N vs SSA. Rectal polyp. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Lower Oesophagus". Two mucosal fragments measuring 2-5 mm. A1. 2. Specimen labelled "GOJ". Three mucosal fragments measuring 3-4 mm. A1. 3. Specimen labelled "Gastric bx". Three mucosal fragments measuring 2-4 mm. A1. 4. Specimen labelled "Duodenum". Two mucosal fragments measuring 2-3 mm. A1. 5. Specimen labelled "Transverse Colon Polyp". Multiple polypoidal mucosal fragments measuring 10 x 6 x 3 mm in aggregate. A1. Please note: Associated vegetable matter not processed. 6. Specimen labelled "Caecal Polyp". Three tan polypoid fragments measuring 2-3 mm. A1. 7. Specimen labelled "Random Colon bx". Three mucosal fragments measuring 1-2 mm. A1. 8. Specimen labelled "Rectal Polyp". A tan polypoid fragment measuring 5 x 3 x 2 mm. Processed whole. A1. **MICROSCOPIC DESCRIPTION** 1. The biopsy shows two fragments of squamous epithelium with minimal attached stroma. No active inflammation, organisms, intestinal metaplasia, dysplasia or malignancy is identified. 2. The biopsy examined at deeper levels shows three fragments of squamocolumnar mucosa with chronic inflammatory cell infiltrate in the lamina propria including plasma cells, and very occasional neutrophils within the lamina propria. No goblet cells indicative of intestinal metaplasia are identified. The glands demonstrate reactive changes however no dysplasia or malignancy is identified. 3. The biopsy shows three fragments of specialised gastric mucosa. No active inflammation or Helicobacter pylori like organisms are seen. There is no evidence of intestinal metaplasia, dysplasia or malignancy. 4. The biopsy shows two fragments of duodenal mucosa with villous architecture and Brunner's glands. There is no villous blunting. The numbers of intraepithelial lymphocytes are predominantly within normal limits but focally and mildly increased. No active inflammation or organisms are seen. There is no evidence of ulceration, granuloma formation, dysplasia or malignancy. 5. Sections show multiple fragments of colonic mucosa, multiple fragments demonstrating serrated, hypermucinous glands. There are areas of dilated crypt bases. No convincing dysplasia is identified. There is no evidence of malignancy. 6. Sections show colonic mucosa with serrated hypermucinous glands with dilated crypt bases. There is no evidence of dysplasia or malignancy. 7. The biopsy shows three fragments of colonic mucosa with preserved crypt architecture. The lamina propria contains a normal complement of chronic inflammatory cells.

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No active inflammation is seen. There is no subepithelial collagen plate thickening or increased intraepithelial lymphocytes. There is no evidence of ulceration, granuloma formation, organisms, dysplasia or malignancy. 8. The biopsy shows a polypoid fragment of colonic mucosa with serrated, hypermucinous glands. No definite dilated crypt bases or lateral extension along the muscularis mucosae is noted. There is no evidence of dysplasia or malignancy. **PATHOLOGIST'S OPINION 1. Lower oesophagus: Within normal limits. 2. GOJ: Squamocolumnar mucosa with chronic inflammation and focal activity, no evidence of goblet cell intestinal metaplasia. 3. Gastric bx: Within normal limits. 4. Duodenum: Mild non specific changes. 5. Transverse colon polyp: Sessile serrated lesion. 6. Caecal polyp: Sessile serrated lesion. 7. Random colon bx: Within normal limits. 8. Rectal polyp: Hyperplastic polyp.** Pathologist: Dr Joan Wang

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